Joseph Medical Clinic

59212 Weber Prairie
Baytown, TX
Phone 346-555-9847

Itemized Medical Bill

Patient:	Emily Miller
Date of birth:	1971-08-20
Insurance claim:	C-1446
Date of injury:	2024-03-24
First visit:	2024-03-24
Last visit:	2024-06-25
Number of visits:	15

Services

Service	Units	Charge
99203 New patient evaluation	1	$369.84
97110 Therapeutic exercise	1	$135.11
97124 Massage therapy	6	$602.95
72040 X-ray, cervical spine	3	$1,554.64
20552 Trigger point injection	4	$2,112.03
TOTAL		$4,774.57

Recent visits

Date	Service	Charge
2024-05-30	97124 Massage therapy	$90.50
2024-06-02	20552 Trigger point injection	$530.10
2024-06-08	72040 X-ray, cervical spine	$602.17
2024-06-15	97110 Therapeutic exercise	$135.11
2024-06-21	97124 Massage therapy	$108.96
2024-06-25	20552 Trigger point injection	$454.56
2024-06-25	97112 Neuromuscular re-education	$92.09

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Treatment History and Summary

Patient:	Emily Miller
Treating provider:	Charles Hensley, DC
Diagnosis:	whiplash

Treatment history by month

Month and services	Visits	Charges
March 2024: evaluation, massage therapy, pain management	3	$1,157.75
April 2024: massage therapy, pain management	3	$623.62
May 2024: imaging, massage therapy	4	$1,162.30
June 2024: imaging, massage therapy, pain management, physical therapy	5	$1,830.90

Treatment summary

Clinical Treatment Summary

The patient presented following a low-speed rear-end collision in a parking lot, reporting neck pain, stiffness, and headache consistent with whiplash-type cervical strain. The mechanism of injury described is plausible for a collision of this nature. Imaging and physical examination findings clearly confirm soft tissue injury to the cervical region. The patient's history includes a minor starin in the same area many years ago, which resolved completely without lasting limitation. Over the course of fifteen visits, care included manual therapy, range-of-motion exercises, and a home stretching program. Treatment followed a typical progression, with frequency tapering as symptoms improved and range of motion returned toward baseline. At the most recent visit, the patient reported minimal residual discomfort and full return to daily activities. Two similar billing entries recorded on the same day are currently under review for accuracy. Discharge from active care is anticipated with a home exercise program and as-needed follow-up.